Zdravdoma Platform

Development Scope and Estimation Brief

Client-facing requirements document for software vendors to prepare timeline, stage plan, team composition, and commercial offer.

Purpose	**Provide one consolidated English brief that a development team can estimate from.**
Primary source	Zdravdoma technical specification (authoritative source for scope and roadmap).
Supporting sources	Website overview / current product baseline and investment-plan context for business priorities.
Expected vendor response	Estimated delivery stages, timeline, team, assumptions, dependencies, and cost proposal.

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1. Document Purpose

This document summarizes the intended Zdravdoma platform in clear English so that a software development company can prepare a formal offer for design, development, QA, deployment, and support. It is written from the client perspective and should be treated as the working basis for estimation, not as a final signed specification.

The Zdravdoma technical specification is the main source of truth for functional scope. The existing website overview helps explain the current baseline and confirms that the platform concept already includes patients, providers, booking, payments, dashboards, and GDPR-aware data handling. The investment-case document helps explain business direction, phased rollout, and monetization priorities.

2. What We Need From the Vendor

- A proposed delivery plan broken into stages or phases.
- Estimated timeline by phase, with major milestones and dependencies.
- Estimated cost by phase and by workstream where possible.
- Recommended team composition and expected roles.
- Technical assumptions, open questions, and items excluded from the estimate.
- Recommended MVP boundary versus later-scale features and integrations.

3. Product Vision

Zdravdoma is a home healthcare platform that connects patients and families with verified healthcare providers who deliver care at the patient's home. The platform is intended to support booking, provider matching, visit execution, reporting, payments, subscriptions, notifications, and healthcare-system integrations. The primary market is Slovenia, with future multi-language and regional growth potential.

4. Proposed Platform Components

- Patient application: iOS and Android mobile app built with React Native, plus responsive web experience.
- Provider application: iOS and Android mobile app built with React Native, plus web dashboard.
- Admin / operations web application: management of users, bookings, providers, content, analytics, and support workflows.
- Backend platform: API gateway, core services, database, storage, notifications, analytics, CMS, and integrations.
- Integration layer: national healthcare systems, reimbursement/billing systems, suppliers, and EHR / hospital systems.

5. Target User Roles

Role	Who they are	Main goals in the platform
Patient / Family	End users booking home healthcare services	Register, manage profile and documents, book services, track visits, pay, review providers, view history.
Provider	Nurse, physiotherapist, doctor,	Get verified, set availability, receive

Role	Who they are	Main goals in the platform
	speech therapist, occupational therapist, dietitian, psychologist, or similar professional	bookings, navigate to visits, complete structured forms, generate reports, see earnings.
Admin / Operations	Back-office staff and dispatch / support team	Approve providers, supervise bookings, manage content and pricing, monitor KPIs, handle escalations, audit data access.
Doctor / Clinical reviewer	On-call doctor or supervising clinician	Receive escalations, review reports, issue prescriptions/referrals where applicable.
Partner systems	NIJZ / ZZZS / suppliers / EHR systems	Exchange insurance, visit, referral, billing, stock, and clinical-document data.

6. Functional Scope

6.1 Patient App / Patient Web

- Registration with email or phone and password, or SSO with Apple and Google.
- SMS OTP verification during onboarding.
- Patient profile including first name, last name, date of birth, address, phone, insurance details, allergies, chronic conditions, and emergency / contact person data.
- Optional national ID / EMŠO storage where required for insurance or integrations.
- Medical document upload (PDF or image), including discharge reports, X-rays, lab results, and OCR extraction of key fields such as diagnosis, date, and doctor.
- Multiple patient addresses with GPS validation for matching and routing.
- Payment methods: card payment via Stripe, invoice / UPN, and future direct insurance payment when integrated.
- Service catalog with individual services and care packages, with filters by category, price, and rating.
- Package comparison view (for example S / M / L packages) with add-on options.
- Real-time calendar booking with date and time-window selection.
- Matching algorithm that proposes the most suitable provider based on distance, specialization, rating, availability, and language.
- Patient can view suggested provider profile and request a replacement.
- Confirmed-booking communications via push, email, and SMS, with reminders 24 hours and 1 hour before visit.
- Cancellation policy handling: free cancellation above the configured threshold; reduced refund / fee below threshold.
- Live visit status such as provider on the way, ETA, visit in progress, and completed. GPS tracking should be opt-in.
- Visit summary after completion, including clinical notes, vital signs, findings, and recommendations when relevant.
- Provider rating and review after the visit.
- Suggested next appointment / repeat booking flow.
- Patient booking history, profile, document center, payment history, premium subscription area, settings, and support.

6.2 Provider App / Provider Dashboard

- Provider registration by role and profession.
- Document upload for license, diploma, insurance policy, identity document, and other verification records.
- OCR plus manual review for provider verification.
- Background-check workflow and status transitions such as pending, verified, active, rejected, or suspended.
- Provider profile with specialization, languages, coverage radius, experience, photo, and biography.
- Digital contract signing (for example DocuSign or internal alternative).
- Availability calendar with recurring and one-time time slots, plus blocked times.
- Calendar synchronization with Google / Apple / CalDAV where feasible.
- New booking inbox with accept / reject actions, auto-accept option, and full booking detail.
- Navigation support via Google Maps / Waze deep linking and route optimization for multiple visits.
- Structured visit forms by service type, including nursing-specific and physiotherapy-specific data capture.
- Photo upload during visit, vital-sign recording, wound assessment, functional tests, and home-exercise planning as applicable.
- Automatic PDF report generation after visit completion.
- Ability to send report to patient and operator / doctor, and later to integrated healthcare systems.
- Patient e-signature capture on tablet or mobile device.
- Urgent escalation flow with red-button concept for serious clinical findings.
- Earnings dashboard showing completed visits, earnings, bonuses, ratings, and payout schedule.

6.3 Admin / Operations

- Admin authentication and role-based access control.
- User, patient, and provider management with verification workflows.
- Booking management, manual interventions, cancellation handling, and dispatch oversight.
- Service catalog, packages, pricing rules, content pages, FAQs, and localization management.
- Monitoring of escalations, notifications, delivery logs, and operational exceptions.
- Analytics and KPI dashboard for business, quality, and growth metrics.
- GDPR workflows including data export, delete requests, consent records, and audit trail access.
- Support tooling for chat / phone workflow references if implemented later.

6.4 Commercial and Billing Features

- Card payments through Stripe.
- Invoice / UPN flow.
- Refund, cancellation fee, and commission logic.
- Provider payout scheduling.
- Insurance reimbursement support through downloadable PDF claim documents in MVP.
- Premium subscription support for patients and potentially providers.
- Future support for direct insurance billing and public reimbursement integrations.

7. Suggested Primary Screens / User Journeys

Area	Core screens / journey
Patient	Home → service catalog → package detail → booking calendar → provider suggestion → confirmed booking → live status → visit summary → rating → history

Area	Core screens / journey
Patient secondary	Profile → documents → payments → subscription → settings → help / support
Provider	Dashboard → calendar → inbox → booking detail → navigation → visit form → report → earnings
Provider secondary	Profile → documents → ratings / feedback → education / training → settings → support
Admin	Login → operations dashboard → provider verification → booking queue → service / pricing CMS → analytics → audit / compliance → support tools

8. Backend and Technical Architecture

The vendor may propose a different implementation, but the source specification suggests the following target architecture direction:

- Backend stack: Node.js / NestJS.
- Primary database: PostgreSQL.
- Cache / real-time locking: Redis.
- Document storage: S3-compatible object storage.
- Search: Elasticsearch where needed.
- Infrastructure: AWS or another GDPR-compliant EU hosting environment such as Hetzner.
- CI/CD: GitHub Actions or equivalent.
- Monitoring and error tracking: Grafana and Sentry or equivalent.
- Apps and admin web should expose secure APIs through an API gateway pattern.
- Auditability, encryption at rest, consent management, and immutable health-data access logs are required.

Proposed core backend modules:

Module	Purpose	Notes
Auth service	Registration, login, MFA, JWT refresh, RBAC	Patient, provider, admin, superadmin roles.
User service	Profiles, documents, verification, GDPR actions	Patient and provider master data.
Booking service	Booking CRUD, status machine, cancellations	Requested → confirmed → in progress → completed → rated.
Matching engine	Geo query, scoring, slot locking	Distance + specialty + rating + load balancing.
Visit service	Structured forms, vitals, media upload, signatures, PDF reports	Clinical visit execution layer.
Billing service	Payments, invoices, refunds, commission, payouts	Stripe + invoice / UPN + future insurance billing.
Notification service	Push, SMS, email, reminders	FCM/APNs + SMS gateway + email provider.
Analytics service	Events, KPIs, data warehouse / dashboards	Operational and growth analytics.
CMS service	Services, packages, pricing, content, localization	Supports business team updates without code release.

Module	Purpose	Notes
Integration service	Healthcare system, reimbursement, supplier, EHR integrations	Phased rollout recommended.

9. Data, Security, and Compliance Requirements

- GDPR-compliant data processing and consent capture.
- Role-based access control for patient, provider, operator, admin, and superadmin access levels.
- Encryption at rest for clinical data and documents.
- Immutable audit logging for profile changes and access to health data.
- Data export and delete workflows, with anonymization / soft-delete logic as required by law and medical-record retention rules.
- Retention of health documentation for the required period (source spec indicates 10 years).
- Daily encrypted backups with disaster-recovery targets to be proposed by vendor.
- Secure handling of OTP, sessions, tokens, certificates, and healthcare-system credentials.
- Consent management for analytics, marketing, insurer sharing, and EHR / national system sharing.

10. Reporting, Metrics, and Analytics

The admin panel should provide operational and business KPIs. The source material specifically highlights the following metric groups:

- DAU / MAU and bookings per day.
- Registration-to-first-booking conversion.
- Confirmed booking to completed visit rate.
- Median time from booking to provider confirmation.
- Patient and provider NPS.
- Average visit rating.
- CAC, LTV, and LTV/CAC ratio.
- Patient and provider churn.
- Repeat visit rate.
- MRR and gross margin.
- Adverse event rate and number of escalations.

Event tracking should cover key actions such as screen views, booking creation, visit start, rating submission, and payment completion. The vendor should recommend the preferred analytics implementation and reporting stack.

11. External Integrations and Phased Roadmap

Integration complexity is one of the most important estimation areas. The development offer should clearly separate MVP work from scale and full-integration phases.

Integration	Business purpose	MVP / later phase	Notes for estimation
eZdravje / NIJZ	Insurance check, prescription, referral, patient summary, electronic visit record	Phase 1–3 depending module	Requires provider registration, certificates, sandbox access, security review, and likely non-trivial certification effort.

Integration	Business purpose	MVP / later phase	Notes for estimation
BIRPIS / ZZZS billing	Direct reimbursement and claim submission for covered services	Phase 2+	Dependent on concession / payer agreements; MVP can generate PDF reimbursement documentation.
Medical suppliers	Inventory tracking and material ordering per provider	Manual in Phase 1, API in Phase 2	Needs supplier API availability and mapping of services to supplies.
Better / openEHR / hospital systems	Read discharge info, write visit report back to EHR, discharge-triggered referral flows	Manual PDF in MVP, pilot in Phase 2, advanced triggers in Phase 3	Likely one of the largest integration risks; should be estimated separately with assumptions.
Maps / routing	Navigation and geolocation	MVP	Google Maps / Waze links, routing APIs, and address normalization.
Payments	Card, invoice, future insurer / public payment flows	MVP + later extensions	Stripe first, then insurance and public payer expansion.
Communications	Push, SMS, email notifications	MVP	Requires gateway/provider selection and message-template management.
Calendar sync	Provider calendar synchronization	Optional MVP or Phase 2	Depends on desired depth of Google / Apple / CalDAV synchronization.

12. Recommended Estimation Structure for the Vendor

To make proposals comparable, we ask the vendor to estimate using the following structure:

1. Discovery and solution design: workshops, backlog definition, architecture, UX flows, technical specification refinement.
2. Phase 1 – MVP: patient app, provider app, admin panel, core backend, booking, matching, payments, notifications, reporting, GDPR basics, and manual workflows for complex integrations.
3. Phase 2 – Scale: improved automation, reimbursement integration, supplier APIs, richer analytics, expanded CMS, improved provider tooling, and selected healthcare-system pilots.
4. Phase 3 – Full integration: advanced eZdravje modules, Better / EHR integrations, referral triggers, direct payer flows, and other enterprise-grade extensions.
5. QA, security testing, release management, infrastructure setup, monitoring, and production deployment.
6. Post-launch support and maintenance options.

13. Items the Vendor Should Explicitly Price or Clarify

- UX / UI design effort versus development only.
- Whether web and mobile are fully included for both patient and provider sides.
- Whether admin and operations tooling is included in the same estimate.
- Effort for OCR, PDF generation, e-signature capture, and document handling.

- Real-time booking / slot locking complexity.
- Push, SMS, and email implementation effort and third-party costs.
- Infrastructure setup, DevOps, environments, and monitoring.
- Security, audit logging, and GDPR implementation depth.
- Integration discovery versus implementation for each external system.
- Testing scope: manual QA, automated testing, load / security testing.
- Warranty / bug-fix period and ongoing maintenance pricing.
- Third-party license costs and pass-through usage fees.

14. Assumptions and Open Questions

The vendor should validate these items during discovery, because they will materially affect price and timing:

- Final MVP boundary: which integrations are included in the first release, and which remain manual or deferred.
- Whether a current codebase / website should be reused, rebuilt, or only used as UX / process reference.
- Exact supported service catalog at launch and number of visit-form templates required.
- Required languages at launch and how localization will be managed.
- Exact provider verification and background-check process, including any third-party services.
- Whether telemedicine, chat, or hotline features are in scope for the first release.
- Who owns clinical protocol definitions and content population for forms, CMS pages, and packages.
- Legal readiness for NIJZ / ZZZS / hospital integrations and whether client will supply necessary agreements, certificates, sandbox access, and schemas.
- Exact payment, payout, and invoicing flows for private and reimbursed care.
- Performance expectations, service levels, and support hours after launch.

15. Deliverables Expected From the Vendor

Deliverable	Expected content
Commercial proposal	Total estimated cost, payment plan, and pricing model.
Delivery roadmap	Phases, milestones, timeline, dependencies, and go-live assumptions.
Scope breakdown	Included modules, integrations, exclusions, and optional items.
Team proposal	Suggested roles, seniority, and approximate allocation.
Technical proposal	Architecture recommendation, stack, infrastructure, security, and QA approach.
Risk log	Main risks, assumptions, and client-side prerequisites.
Support proposal	Post-launch maintenance, SLA, warranty, and change-request handling.

16. Suggested High-Level Delivery Stages

This section is intentionally indicative only. The vendor is expected to confirm or improve it in the proposal.

Stage	Goal	Typical outputs	Comments
0. Discovery	Validate scope and assumptions	Workshops, backlog, technical architecture, UX map, estimation update	Should reduce uncertainty before full build commitment.
1. MVP Build	Launch private-pay platform with manual workaround paths	Core apps, admin, backend, payments, notifications, reports, analytics basics	Recommended baseline for first formal estimate.
2. Operational Scale	Improve automation and business efficiency	Expanded analytics, CMS maturity, provider workflows, supplier and reimbursement integrations	Can follow real usage after MVP launch.
3. Full Healthcare Integration	Connect deeply into national and hospital ecosystems	Advanced NIJZ / ZZZS / EHR integrations and referral automations	Best treated as separate work packages because of external dependencies.

17. Summary for Estimation

In summary, we are seeking a software partner to build a multi-sided home healthcare platform with patient and provider applications, an admin and operations panel, a secure backend, payment and notification systems, clinical visit reporting, analytics, and a phased path toward public-health and hospital integrations. The estimate should clearly distinguish the cost and timeline of the MVP from later scaling and integration phases.

The proposal should be practical, commercially clear, and explicit about assumptions. If the vendor recommends narrowing or re-sequencing the scope for a stronger MVP, that recommendation is welcome as long as it includes rationale and impact on cost and timing.

Appendix A – Source-Based MVP vs Later-Phase Scope

Area	MVP expectation	Later-phase expectation
Patient & provider apps	Core onboarding, profiles, booking, matching, visit execution, reporting, payments, notifications	Broader automation, richer subscriptions, advanced support, more service templates
Payments	Stripe + invoice / UPN	Direct insurance and public payer flows
eZdravje	At minimum insurance check and only feasible starter integration if legally ready	Prescription, electronic record write-back, patient summary, full integration
BIRPIS	PDF reimbursement support	XML or real-time reimbursement submission
Suppliers	Manual ordering and refund workflow	API ordering, stock sync, replenishment automation
Better / hospital systems	Manual discharge-document upload	Pilot integration, then discharge-triggered referral automation
Analytics	Operational dashboard and event tracking	Warehouse-grade reporting, cohort analysis, finance and growth dashboards

Appendix B – Business Context Useful for Estimation

- Primary launch market is Slovenia.
- There is future intent for multi-language support and regional expansion.
- The business model may include visit commissions, subscriptions, and partner integrations.
- The platform is expected to support both private-pay use cases and future reimbursed / insured flows.
- The vendor should therefore design for scale and integrations, even if the first delivery phase is narrower.